

Introduction

Mental health terms have become embedded in our daily communication. People routinely describe themselves as “traumatized” by an awkward social moment, “OCD” about keeping things clean, and “depressed” after a stressful week. On social media, short videos listing symptoms of ADHD or anxiety can quickly gain millions of views, while formal clinical diagnoses are rapidly rising in schools, clinics, and workplaces. While some view this trend as a long-overdue recognition of human suffering historically silenced by stigma and shame, others argue it signals that we are treating normal human struggles as medical problems.

In evaluating this debate, I argue that the current mental health system is highly effective at recognizing distress, but structurally ineffective at responding to it correctly. By applying the Complete State Model of Mental Health, Foulkes’ theory of “concept creep,” and the sociology of biographical disruptions, this essay will demonstrate how our social institutions frequently encourage overdiagnosis in minor cases while simultaneously neglecting severe psychiatric illnesses, turning everyday distress into medicalized identities.

Redefining Recognition: The Theoretical Framework

To understand how a system can be simultaneously hyper-aware yet structurally ineffective, we must first separate two different concepts: misdiagnosis and overdiagnosis. Misdiagnosis occurs when a doctor gives a patient the wrong label by mistake, such as confusing a physical thyroid condition for clinical depression. Overdiagnosis, on the other hand, happens when a person technically fits the official checklist for a medical condition, but the label does not actually benefit them. It applies psychiatric terms to mild distress that would have resolved on its own, or it leads to treatments that cause more harm than good (Thombs et al., 2019).

For decades, public awareness campaigns have operated under the assumption that mental health is simply the absolute absence of mental illness. This essay challenges that baseline by utilizing the Complete State Model of Mental Health, which conceptualizes mental illness and positive mental well-being as two completely separate axes (Nalipay et al., 2024).

Under this model, a person can live without a diagnosable disorder but still experience a profound sense of emptiness, while an individual with a diagnosed condition can achieve a high quality of life given the correct structural support. Currently, our cultural recognition is predominantly one-sided. By focusing primarily on clinical deficits rather than teaching individuals the mechanisms of positive well-being, our institutions optimize for the identification of pathology while frequently failing to foster actual psychological resilience.

Overdiagnosis Begins in the Classroom

This institutional preoccupation with deficit tracking heavily impacts our schools, serving as the primary incubator for modern diagnosis. Recently, educators have received substantial training in Mental Health Literacy (MHL). However, because this training focuses almost exclusively on identifying pathology, teachers are conditioned to spot signs of disorders like ADHD or anxiety while remaining unequipped to cultivate a student's positive well-being (Nalipay et al., 2024). When institutions train educators to look only for dysfunction, normal behavioral variations are naturally viewed through a medical lens.

Attention-Deficit/Hyperactivity Disorder (ADHD) provides a clear example of this overdiagnosis loop. In a comprehensive systematic scoping review of 334 published studies, researchers found convincing evidence of widespread ADHD overdiagnosis, driven heavily by a “relative age effect” in schools (Kazda et al., 2021). The data reveals that the youngest children in a school year are more likely to be diagnosed with ADHD than their older classmates. This suggests that normal childhood immaturity is frequently medicalized due to inflexible classroom demands. For instance, a seven-year-old child who cannot sit still for six hours a day is often just displaying natural developmental variance. However, because modern classrooms demand constant focus and quiet conformity, this natural behavior is treated as an internal brain disorder.

Ultimately, the researchers introduced a “hypothesis of diminishing returns” to explain the fallout of this trend (Kazda et al., 2021). They demonstrated that for children on the milder end of the symptom spectrum, the institutional and clinical harms of a diagnosis can outweigh the clinical benefits. These documented harms range from a sense of internal passiveness to medication-induced irritability and a documented decrease in overall happiness. If educators were instead trained in Positive Mental Health Literacy, they could improve a student's life satisfaction and creative thinking through regular classroom adjustments. Without this dual framework, schools risk turning manageable behavioral differences into permanent medical labels, exposing mild cases to real physical risks of psychiatric medications—such as insomnia, loss of appetite, or slowed growth—for little long-term clinical reward (Kazda et al., 2021).

This imbalance reveals a broader limitation of modern mental health systems. As Sampaio et al. argue in a critical evaluation of healthcare literacy models, expanding recognition without expanding meaningful support risks turning diagnosis into an endpoint rather than the beginning of genuine recovery (Sampaio et al., 2022). When schools prioritize identifying deficits over adapting environments, psychiatric labels may function more as administrative tools than pathways to care.

Concept Creep and Social Media Descriptions

The deficit-driven vocabulary constructed in the classroom does not remain confined to school walls. Rather, it provides the foundational language that digital environments exploit. Beyond the

classroom, social media functions as another powerful force for these diagnoses because clinical evaluation is not a simple medical test, but a subjective verbal interaction between a patient and a doctor. Since doctors cannot see raw emotions, they must entirely rely on the stories patients present to them.

In a conversation analytic study of primary care encounters, sociologist Alexandra Tate demonstrated that physicians are generally hesitant to hand out psychiatric labels (Tate, 2019). When patients present mental health issues by merely listing their raw, vague physical symptoms like headaches or tiredness, doctors systemically default to providing a physical health diagnosis and ordering standard blood tests. Conversely, physicians usually cross the boundary into a psychiatric diagnosis when a patient provides a concrete narrative linking their symptoms to a “biographical disruption,” clear proof that a symptom is actively ruining a patient’s ability to handle their daily life (Tate, 2019). If a patient articulates their story from a framework of life-disrupting impairment such as stating that their anxiety prevents them from working or leaving the house, a psychiatric diagnosis is immediately legitimized.

Social media alters this clinical interaction by rewriting how young people interpret and narrate their inner lives. As noted by psychologist Dr. Lucy Foulkes, this shift is driven by “concept creep,” a phenomenon where psychological terms once reserved for severe illness expand to cover much milder, everyday experiences (Foulkes, 2026). Short videos on digital platforms turn specialized psychiatric language into ordinary self-description, encouraging users to view normal experiences—such as procrastinating, daydreaming, or being sensitive—as symptoms of major psychiatric disorders (Lewis, 2022). Consequently, many patients are learning to talk about themselves using precise clinical terminology, arriving at doctor appointments with a pre-packaged self-diagnosis that exploit the exact “biographical disruption” narratives physicians look for.

This suggests that the apparent rise in mental health awareness may partially reflect a shift in vocabulary rather than purely an expansion of underlying pathology. The more our culture encourages people to interpret themselves through diagnostic categories, the more difficult it becomes to distinguish clinical disorder from ordinary emotional distress.

Furthermore, our social institutions have created powerful structural rewards for holding these medical labels. In schools, a diagnosis functions as the gateway to gain extra testing time, fewer assignments, or special support. In workplaces, a formal medical document is required to ask for flexible hours or protection from being fired. In the mind, a label can provide immediate comfort, convincing the individual that their struggles are a medical condition rather than a personal failure (Lewis, 2022). As a result, our social structures actively incentivize individuals to turn passing life challenges into permanent medical identities.

To be sure, expanded diagnostics have historically thrown vital light on genuine, long-ignored suffering. As documented in a review of healthcare disparities, historical under-recognition penalized racial minorities and women who long faced systemic gaps in clinical access and validation for severe disorders (McGuire and Miranda, 2008). Decreased stigma and expanded tracking have thrown open the doors to legitimate therapeutic care for these populations. However, acknowledging that the diagnostic net has widened to help the historically neglected does not mean we must ignore that same net now overextends to pathologize transient human distress.

The Global Paradox of Wealth and Neglect

When a culture successfully trains its citizens to interpret ordinary distress through the lens of disruptive psychiatric impairment, institutional attention inevitably follows the most articulate consumers. Ironically, while mild emotional distress is being over-medicalized, severe mental illness is deeply neglected, creating a global “paradox of mental health” (The Paradox, 2013). In this system, mild problems are widely over-diagnosed and over-treated. Meanwhile, there is a severe lack of basic care for those who are suffering the most. This resource imbalance directly answers the essay question: society is often recognizing distress more readily while allocating disproportionate attention to profitable wellness markets and insufficient resources to severe psychiatric crises.

During the pandemic, US venture capitalists invested \$5.1 billion into digital mental health tools like wellness apps, mindfulness trackers, and online therapy platforms that primarily helped consumers manage everyday stress instead of helping individuals with severe illnesses like chronic schizophrenia, bipolar disorder, or deep addiction (Insel, 2022).

According to empirical data from the Prison Policy Initiative, roughly 43% of individuals held in state prisons have a diagnosed mental condition (Research on the Prevalence). This statistic illustrates how structural neglect actively channels chronic psychiatric crises out of medical infrastructure and directly into the carceral state. Furthermore, the World Health Organization’s mental health gap action programme update (mhGAP) emphasizes that in low and middle income regions, massive treatment gaps for severe psychiatric conditions remain completely unaddressed because specialized medical clinics are fundamentally inaccessible to impoverished populations (Lorenz et al., 2024). This uneven allocation of resources demonstrates that society’s current standard of “recognition” is a market-driven system that over-recognizes comfortable, corporate-backed wellness while ignoring raw psychiatric crises.

This structural disparity scales up dramatically when analyzing international healthcare systems. While high-income countries debate whether ordinary stress qualifies as an illness, developing regions face severe healthcare shortages. Millions of individuals remain untreated for severe

depression, trauma, or psychosis because services are inaccessible. Yet, these low-resource environments provide a valuable framework for decentralized care, proving that mental health support can be integrated directly into local communities rather than relying on specialized medical institutions. In Pakistan, for example, “Lady Health Workers” were trained to support depressed patients in local clinics, integrating mental healthcare into local communities (Goldberg, 2000). Instead of relying only on expensive, distant specialists, this model builds mental support directly into everyday neighborhood care. Wealthier countries frequently miss this lesson because they are too focused on isolated, expensive medical clinics.

Conclusion

Society has become increasingly skilled at identifying psychological distress, yet the institutions built to respond to that distress often medicalize manageable suffering while neglecting severe psychiatric illness. The current rise in mental health diagnoses is not solely a sign that society has become better at recognizing disease. Instead, it demonstrates that individuals have learned to interpret and describe their situational difficulties through an individualized model that treats structural challenges as internal brain defects. When our social structures reward these medical labels with academic advantages, workplace protections, and personal reassurance, separating true clinical disorders from normal human struggles becomes exceptionally difficult.

The internet has amplified this problem by exposing young people to simplified symptom checklists and self-diagnostic content. This superficial obsession with light wellness has become a powerful tool for neglecting severe psychiatric conditions.

To restore our focus, a systemic shift toward positive mental health literacy is necessary. Improving existing pathways of education and care will help communities identify the environmental and structural roots of their distress, encouraging us to fix our surroundings rather than merely medicating the individual. Becoming aware of our entrenched institutional habits and one-sided thinking mechanisms is valuable, as it enables us to respond to human suffering with more clarity and fairness to better our world.

Works Cited

- Foulkes, Lucy. "Are We Really Overdiagnosing Mental Illness?" *The Guardian*, 22 Feb. 2026, www.theguardian.com/books/2026/feb/22/are-we-really-overdiagnosing-mental-illness. Accessed 11 Mar. 2026.
- Goldberg, David. "Expanding and Strengthening the Mental Health Care System." *Bulletin of the World Health Organization*, vol. 78, no. 4, Apr. 2000, p. 513. *Gale in Context: Science*, link.gale.com/apps/doc/A63124550/SCIC?u=mlyn_n_governor&sid=bookmark-SCIC&xid=a3c39396. Accessed 2 June 2026.
- Insel, Thomas. "Mental Health Care 2.0." *Science*, vol. 376, no. 6596, 27 May 2022, p. 899, <https://doi.org/10.1126/science.add1020>. Accessed 9 Mar. 2026.
- Kazda, Luise, et al. "Overdiagnosis of Attention-Deficit/Hyperactivity Disorder in Children and Adolescents." *JAMA Network Open*, vol. 4, no. 4, 12 Apr. 2021, p. e215335, <https://doi.org/10.1001/jamanetworkopen.2021.5335>. Accessed 9 Mar. 2026.
- Lewis, Ralph. "The Curious Cachet of a Psychiatric Diagnosis." *Psychology Today*, vol. 55, no. 1, 1 Jan. 2022, pp. 48-49. *Science Reference Source*, research.ebsco.com/linkprocessor/plink?id=4dc51382-7427-327b-90f8-2b54372054a5. Accessed 2 June 2026.
- Lorenz, Carolin, et al. "Update of the World Health Organization's Mental Health Gap Action Programme Guideline for Psychoses (Including Schizophrenia)." *Schizophrenia Bulletin*, vol. 50, no. 6, 3 May 2024, pp. 1310-25, <https://doi.org/10.1093/schbul/sbae043>. Accessed 7 June 2026.

- McGuire, Thomas G., and Jeanne Miranda. "New Evidence regarding Racial and Ethnic Disparities in Mental Health: Policy Implications." *Health Affairs*, vol. 27, no. 2, Mar. 2008, pp. 393-403, <https://doi.org/10.1377/hlthaff.27.2.393>. Accessed 7 June 2026.
- Nalipay, Ma Jenina N, et al. "Positive Mental Health Literacy for Teachers: Adaptation and Construct Validation." *Current Psychology*, vol. 43, no. 6, Feb. 2024, p. 4888+. *Gale in Context: Science*, [dx.doi.org/10.1007/s12144-023-04694-y](https://doi.org/10.1007/s12144-023-04694-y). Accessed 12 Mar. 2026.
- "The Paradox of Mental Health: Over-treatment and Under-recognition." *PLoS Medicine*, vol. 10, no. 5, May 2013. *Gale in Context: Science*, [dx.doi.org/10.1371/journal.pmed.1001456](https://doi.org/10.1371/journal.pmed.1001456). Accessed 12 Mar. 2026.
- "Research on the Prevalence and Treatment of Mental Illness in the Criminal Legal System." *Prison Policy Initiative*, 27 May 2026, www.prisonpolicy.org/research/mental_health/. Accessed 7 June 2026.
- Sampaio, Francisco, et al. "Mental Health Literacy: It Is Now Time to Put Knowledge into Practice." *International Journal of Environmental Research and Public Health*, vol. 19, no. 12, 8 June 2022, p. 7030, <https://doi.org/10.3390/ijerph19127030>. Accessed 7 June 2026.
- Tate, Alexandra. "Matter over Mind: How Mental Health Symptom Presentations Shape Diagnostic Outcomes." *Health: An Interdisciplinary Journal for the Social Study of Health, Illness and Medicine*, vol. 24, no. 6, 3 Apr. 2019, pp. 755-72, <https://doi.org/10.1177/1363459319834991>. Accessed 9 Mar. 2026.
- Thombs, Brett, et al. "Defining and Evaluating Overdiagnosis in Mental Health: A Meta-Research Review." *Psychotherapy and Psychosomatics*, vol. 88, no. 4, 2019, pp. 193-202, <https://doi.org/10.1159/000501647>. Accessed 9 Mar. 2026.